

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Foundayo (orforglipron) Tablets

for weight management

NHS funding position and where this PGD sits

- Orforglipron was authorised by the MHRA on 10 August 2026. It is not currently funded on the NHS: NICE is appraising it through a Single Technology Appraisal, Orforglipron for managing overweight and obesity, ID6516.
- Supply under this PGD is therefore a private service. Patients must be told that this is a private supply, what it costs, and that NHS-funded weight management options exist for which they may be eligible.
- The NICE scope names tirzepatide (TA1026), semaglutide (TA875), liraglutide (TA664) and prescription-dose orlistat as comparators, alongside diet and activity alone. Where a patient would meet the criteria for an NHS-funded option, or for referral to a specialist weight management service or bariatric surgery, that should be discussed before a private supply is made.
- NICE guideline NG246 recommends multicomponent intervention as the treatment of choice, with medicines used alongside reduced calorie intake and increased physical activity. This PGD does not replace that: the lifestyle plan is part of the consultation, not an optional extra.

A note on BMI thresholds and ethnicity

- The inclusion criteria below follow the marketing authorisation: BMI 30 kg/m² and above, or 27 to below 30 with at least one weight-related comorbidity.
- NHS pathways differ. NICE guideline NG246 defines obesity from a BMI of 27.5 kg/m² for some ethnic groups, and the weight management technology appraisals reduce their BMI thresholds by 2.5 kg/m² for people from some ethnic minority backgrounds, because the risk of weight-related ill health arises at a lower BMI.
- This is stated so that pharmacists understand the difference rather than assume an error. A patient from one of those groups who falls below the licensed threshold is not eligible under this PGD, but may be eligible for NHS assessment, and should be told so and referred rather than simply declined.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must be competent in weight management consultation, BMI calculation and the recognition of red flags requiring referral • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products

	All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005
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Initial Assessment

Before commencing treatment with weight management therapies, the patient must be assessed by a registered healthcare professional. The initial assessment should be face to face. The assessment should include, but may not be limited to:

- Causes of weight gain. Refer the patient to their GP if prescribed medication is causing weight gain.
- The patient's lifestyle, diet and level of exercise.
- Has the patient tried to lose weight in the past? Discuss what has and has not worked.
- Other factors that may be causing weight gain, including mental health, environmental and psychological factors. Consider signposting to another health professional, for example a mental health team.
- Other disease states that may predispose to weight gain. Consider referring to the GP for further advice.
- The patient's expectations of weight loss, and whether they are realistic for their height and body frame.
- Calculate BMI and determine the ideal weight. Set a realistic target weight and agree review intervals.

Clinical condition or situation to which this PGD applies

PGD Indication	Foundayo (orforglipron) is a once-daily oral GLP-1 receptor agonist indicated in combination with a reduced-calorie diet and increased physical activity to reduce excess body weight and maintain weight reduction long term in adults with an initial Body Mass Index (BMI) of 30 kg/m² or above (obesity), or 27 kg/m² to below 30 kg/m² (overweight) in the presence of at least one weight-related comorbidity. Authorised by the MHRA on 10 August 2026. This medicine is subject to additional monitoring; report all suspected adverse reactions via the MHRA Yellow Card scheme.
Inclusion criteria	- Adults aged 18 to 85 years (inclusive). - BMI 30 kg/m² or above, or BMI 27 kg/m² or above with at least one weight-related comorbidity (for example hypertension, type 2 diabetes mellitus, dyslipidaemia, obstructive sleep apnoea, established cardiovascular disease). - Patient is willing to follow a reduced-calorie diet and increase physical activity in line with the agreed lifestyle plan. - Initial assessment completed and documented (see the Initial Assessment section above). - Informed consent obtained, including off-label considerations where applicable, the side-effect profile and treatment expectations. - No exclusion criteria present. - Patient is able to take one tablet once daily, swallowed whole. Unlike oral semaglutide, Foundayo may be taken with or without food and needs no fasting period or waiting time before other food, drink or medicines.
Exclusion criteria	- Adults under 18 years of age. - Adults aged over 85 years (upper age limit under this PGD; refer to a specialist if treatment is being considered). - Pregnancy, breastfeeding, or planning pregnancy. Effective contraception is required throughout treatment. Orforglipron must be discontinued at least 3 weeks before a planned pregnancy because of its half-life, and stopped immediately if pregnancy occurs or is suspected (SPC section 4.6).

	• Known hypersensitivity to orforglipron or to any of the excipients. • Personal or family history of medullary thyroid carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN 2). • History of pancreatitis, acute or chronic. • Severe gastrointestinal disease, including gastroparesis or severe persistent gastrointestinal disorder. • Current cholelithiasis (gallstones) or cholecystitis, or cholecystectomy within the last 3 months. • Obesity caused by an endocrinological disorder. If the patient was already overweight prior to that diagnosis, this exclusion may not apply. • Concurrent use of any other GLP-1 receptor agonist or insulin secretagogue used for weight management. • Type 1 diabetes mellitus. • Diabetic retinopathy. Treatment may worsen retinopathy; defer or refer to a specialist. • Patients on insulin or sulphonylureas whose GP is unwilling to monitor and adjust their hypoglycaemic regimen. • Severe renal impairment (eGFR below 30 mL/min/1.73 m²) or end-stage renal disease. • Known diagnosis of heart failure with reduced ejection fraction below 40%. • Active eating disorder (anorexia nervosa, bulimia, or binge-eating disorder under specialist care). • BMI below the PGD inclusion threshold. • Patients who, in the clinical judgement of the healthcare professional, are not suitable for the medication. • Severe hepatic impairment (Child-Pugh class C). Orforglipron is not recommended; refer. • Taking a strong CYP3A4 inhibitor that also inhibits OATP1B, for example ritonavir. Concomitant use should be avoided; refer to the GP. • Taking a strong CYP3A4 inducer, for example rifampicin, carbamazepine, phenytoin or St John's wort. Concomitant use should be avoided; refer to the GP.
Cautions	• Counsel on gradual dose escalation and gastrointestinal side effects; consider delaying titration or reducing to the previous dose if significant symptoms occur. • History of suicidal ideation, or active severe mental illness. Ensure appropriate psychiatric oversight is in place, monitor mood at review, and refer if there is any concern. Do not supply where oversight is absent and concern exists. • Mild to moderate renal impairment: monitor for dehydration secondary to gastrointestinal side effects. • Concomitant oral medications: delayed gastric emptying may reduce the absorption of oral medicines, especially those with a narrow therapeutic index. Counsel accordingly. • For individuals who take HRT, due to the lack of data regarding absorption, non-oral products (for example patch, gel, or levonorgestrel intrauterine device) may be considered. • Orforglipron delays gastric emptying and thereby has the potential to impact the rate of absorption of concomitantly

	administered oral medicinal products, especially those with a narrow therapeutic index. Upon initiation of orforglipron treatment in patients on warfarin, frequent monitoring of INR is recommended. • Cases of pulmonary aspiration have been reported in patients receiving GLP-1 receptor agonists undergoing general anaesthesia or deep sedation for surgery. The increased risk of residual gastric content due to delayed gastric emptying should therefore be considered before procedures under general anaesthesia or deep sedation. • Cases of tachycardia have been reported. In patients with a pre-existing increased heart rate, use with caution and seek specialist advice before use. If a patient experiences a clinically relevant sustained increase in resting heart rate, treatment should be discontinued and advice sought. • Counsel on the signs and symptoms of pancreatitis; discontinue and refer urgently if suspected. • Sulfonylurea or insulin co-use in patients with comorbid type 2 diabetes carries a hypoglycaemia risk. Refer to the prescribing GP or specialist before starting. • Risk of dehydration. Counsel on adequate fluid intake during gastrointestinal side effects. • Reassess the benefit-risk balance if there is no clinically meaningful weight loss, typically less than 5%, after 6 months on the maintenance dose. • Oral hormonal contraception. Orforglipron may reduce the efficacy of oral hormonal contraceptives (SPC section 4.5). Advise the patient to switch to a non-oral method, or to add a barrier method, for 30 days after starting orforglipron AND for 30 days after every dose increase. On a six-step titration that is a separate 30 day window each time the dose goes up. Record that this advice was given at initiation and at each escalation. • Hypotension. Orforglipron may lower blood pressure, and hypotension has been reported more frequently in patients already taking antihypertensive medicines. Ask about dizziness, light-headedness and falls at each review, and refer to the GP for review of antihypertensive therapy where symptoms occur. • Simvastatin. The simvastatin dose should be halved when it is taken with orforglipron, because exposure to the active metabolite increases up to 2.5-fold. Do not adjust the statin yourself: refer to the prescriber and do not supply until the position is confirmed. • Rosuvastatin. Caution where the rosuvastatin dose exceeds 20 mg daily, as orforglipron increases rosuvastatin exposure. Refer to the prescriber. • Oral topotecan. Patients taking oral topotecan should be monitored carefully for adverse reactions. These patients are under oncology care; refer rather than supply. • OATP1B inhibitors. Where the patient takes a clinical OATP1B inhibitor, for example ciclosporin, the maximum dose of orforglipron is 9 mg once daily. Where the medicine inhibits both OATP1B and CYP3A4 strongly, for example ritonavir or telaprevir, orforglipron should be avoided altogether and the patient referred. • CYP3A4 interactions: where the patient takes a strong CYP3A4 inhibitor, the maximum dose of orforglipron is 9 mg once daily.
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	Where they take a moderate CYP3A4 inducer, monitor effectiveness and escalate the dose as needed. Check the full medication list at every visit, including over the counter products and St John's wort. • Hepatic impairment: no dose adjustment is needed in mild or moderate impairment (Child-Pugh A and B). Severe impairment (Child-Pugh C) is an exclusion. • Renal impairment and age do not require a dose adjustment for orforglipron, but the gastrointestinal effects and the risk of dehydration still apply, so counsel on fluid intake.
Actions if patient is excluded or declines treatment	• Discuss the reason for exclusion with the patient and ensure they understand. • Advise on alternative treatment options and how these can be accessed, for example the GP, a specialist weight management service, or lifestyle programmes. • If the exclusion relates to an undiagnosed or unmanaged comorbidity, recommend GP review. • Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Foundayo (orforglipron) tablets. Strengths: 0.8 mg, 2.5 mg, 5.5 mg, 9 mg, 14.5 mg and 17.2 mg. Confirm the strengths available under the UK licence against the current SPC before supply. Record the product name and batch number for traceability.
Legal category	POM
Storage	Store in the original container, below 30°C unless the SPC states otherwise. Confirm storage against the current SPC.
Route/method of administration	• Oral, once daily, with or without food, at any time of day. • Swallow the tablet whole. Do not break, crush or chew. • Only one tablet per day. Never combine tablets of lower strengths to make up a higher dose.
Dose and frequency of administration	• Start at 0.8 mg once daily. • After at least 30 days at 0.8 mg, increase to 2.5 mg once daily. • After at least 30 days at 2.5 mg, increase to 5.5 mg once daily. • The dose may then be increased to the next level, 9 mg, then 14.5 mg, then 17.2 mg once daily, after at least 30 days at the current dose, based on response and tolerability. • Maximum dose 17.2 mg once daily. • Escalate no faster than the intervals above, as slower escalation reduces gastrointestinal adverse effects. • Maximum 9 mg once daily where the patient is taking a strong CYP3A4 inhibitor, or a clinical OATP1B inhibitor such as ciclosporin (see cautions). • If a dose is missed, resume dosing as soon as possible. Never take more than one tablet in a day and never double up to make up a missed dose (SPC section 4.2). • In case of significant gastrointestinal symptoms during titration, consider delaying a dose increase or lowering to the previous dose until symptoms have improved. • If the patient wants to recommence treatment after stopping, the dose should be titrated again starting at 0.8 mg. The BMI inclusion criteria for initiation must be applied if more than 2

	months have passed since discontinuing treatment.
Quantity to be administered and/or supplied	- One month of treatment at the current strength per patient appointment. - This PGD does not allow additional medication to be supplied to enable patients to stock up.
Maximum or minimum treatment period	- Reassess clinical benefit, tolerability and target weight at each visit. - If the patient has not lost at least 5% of their initial body weight after 6 months on the maximum tolerated dose, a decision is required on whether to continue treatment. - When the patient reaches their target weight, discuss whether to continue treatment to maintain it.
Adverse effects	Very common and common: nausea, diarrhoea, vomiting, constipation, dyspepsia, abdominal pain, decreased appetite, eructation and fatigue. Also reported: gallstones, hypoglycaemia when used with a sulfonylurea or insulin, and increases in heart rate. Uncommon or rare: acute pancreatitis, cholecystitis and hypersensitivity reactions including anaphylaxis. Refer to the current SPC for the full list, including any boxed warning carried on the UK licence.
Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. This medicine is subject to additional monitoring, so report all suspected reactions.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP with whom the individual is registered - name of healthcare practitioner - name and brand of medication, and batch number - date of supply, dose, form, route and quantity supplied - height, weight and BMI at this visit, and the target weight agreed - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken - that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. Adults aged 18 and over: keep records for 8 years.

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medicine, together with written lifestyle, diet and physical activity advice and the agreed target weight.
Follow-up advice to be given to patient or carer	Explain the expected pattern of weight loss and that the medicine works alongside diet and activity, not instead of them. Counsel on gastrointestinal side effects and how to manage them, on adequate fluid intake, and on the warning symptoms that need urgent attention: severe abdominal pain, persistent vomiting with dehydration, jaundice, sudden visual loss, or a sustained rise in resting heart rate. Advise when to return for review and that treatment will be reassessed if less than 5% of body weight has been lost after 6 months on the maintenance dose.

Key references

Summary of Product Characteristics for the product supplied, current version
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- NICE guidance on obesity management and on the relevant medicine
- MHRA Drug Safety Updates relevant to GLP-1 receptor agonists
- British National Formulary (BNF)
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		21/08/2026

Chris Pilkington	Pharmacist GPhC: 2046322		21/08/2026
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Change history

Version number	Change details	Date
002	Reconciled against the UK Summary of Product Characteristics, which published after version 001 was signed. Version 001 was drafted from Lilly prescribing information because the UK SPC was not yet available, and its change note recorded that the SPC must be checked. Corrections made: the pregnancy exclusion wrongly carried discontinuation intervals for semaglutide and tirzepatide and now states the licensed interval for orforglipron, at least 3 weeks before a planned pregnancy; the interaction between orforglipron and oral hormonal contraceptives was absent and is now a caution requiring non-oral or barrier contraception for 30 days after initiation and after every dose increase; a hypotension caution has been added, noting the higher frequency in patients on antihypertensives; interaction guidance added for simvastatin (halve the dose), rosuvastatin above 20 mg, oral topotecan and clinical OATP1B inhibitors, with the 9 mg ceiling extended to OATP1B inhibitors; the missed dose instruction, which was carried over from injectable GLP-1 practice, now follows the SPC; and a duplicated severe hepatic impairment exclusion has been removed. No change to the titration schedule or the inclusion criteria, both of which were confirmed correct against the SPC.	21st August 2026
001	Development and issue of new PGD following MHRA authorisation of orforglipron on 10 August 2026, requested by PPH after their meeting with Lilly. Inclusions, exclusions and cautions aligned with the Wegovy tablets PGD so the weight management set reads consistently, with the additions specific to orforglipron: dose escalation at 30 day intervals from 0.8 mg to a maximum of 17.2 mg, severe hepatic impairment as an exclusion, and the CYP3A4 interaction handling including the 9 mg ceiling with a strong inhibitor. Dosing drafted from Lilly prescribing information; the UK SPC must be checked before signature. NHS funding position recorded: not NHS funded, NICE appraisal ID6516 in progress, so supply under this PGD is private, with the NICE comparators and the ethnicity BMI difference noted so patients are referred rather than simply declined.	14th August 2026